FEATURE ARTICLE

The Rest Cure Of Silas Weir Mitchell in the 19th Century and Japanese Morita Therapy

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Dr. Lester obtained his Ph.D. from Brandies University where he studied under Abraham Maslow. He has written extensively on death, suicide, and murder. He is currently president of the International Association for the prevention of suicide and his latest book is "Suicide in Creative Women."

The rest cure devised by Silas Weir Mitchell in America in the 19th Century (primarily for depressed and anxious women) and Morita therapy in Japan in the 20th Century (for neurasthenic and anxious patients) are seen as similar in that they encouraged patients who lack self-confidence to conform to the prevailing cultural norms, that is, to be happy with their oppression.

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In the 19th Century, Dr. Silas Weir Mitchell (1829-1914) devised a rest cure for medically and psychiatrically ill patients (Earnest, 1950). He ran a sanitarium near Philadelphia which became famous for treating the nervous disorders of women (Martin, 1887). Women came from all over America to his sanitorium for treatment, including Charlotte Perkins Gilman, who was later to become one of the leading feminists in the nation. Her experience with his treatment revealed features of it which bear a striking resemblance to a form of therapy for nervous disorders which has become popular in Japan in recent years, Morita therapy (Chang, 1974). This essay will describe briefly both Mitchell's rest cure and Morita therapy and then point to the similarities between the two systems of treatment.

Mitchell's Rest Cure

Mitchell's rest cure centered around providing complete rest for the patient, together with systematic feeding and passive exercise through massage and electrical stimulation of the muscles (Mitchell, 1877). Mitchell's theory was that most of the women who came to him for treatment were overly thin and lacked blood (that is, they were anemic). By providing rest and food, his plan was to increase their weight and to remove their anemia.

Mitchell was aware that a variety of factors affected weight (including age, sex, season, climate, diet and genetic predisposition), but he felt that Americans were particularly prone to being underweight. He thought that his treatment was appropriate for medical illnesses such as fevers and pneumonia, but especially suitable for women with nervous exhaustion and hysteria. He describes his typical patients as follows:

> Everything wearies her - to sew, to write, to read, to walk - and by and by the sofa or the bed is her only comfort. Every effort is paid for dearly, and she describes herself as aching and sore, as sleeping ill and awaking unrefreshed, and as needing constant stimulus and endless tonics. Then comes the mischievous role of bromides, opium, chloral, and brandy (pp. 38-39).

This typical patient appears to be depressed. Mitchell noted that his treatment was not for "true melancholics" (in modern terms, those with a major affective disorder) but rather for those who have a "depression of spirits from loss of all hope of relief" (p. 45).

The treatment involved complete seclusion in the clinic. The patient was confined to bed for up to eight weeks. The patients were not allowed to sit up, read, write, sew, or use their hands in any way. Initially, patients were forced to use the toilet while lying down and were fed by their nurse. Patients received a sponge bath three times a week. After a few weeks, patients were allowed

to sit up, feed themselves, and to have the nurse read to them. Mitchell noted that the patients quickly become happy to follow the doctor's mandates.

Diet was important. Mitchell restricted the patients to two quarts of skimmed milk every three hours for at least a week, after which a more regular diet was allowed, with milk remaining an important part. The patients were given a massage for an hour each day, with particular focus on kneading the muscles and pinching the skin, and electrical stimulation of the muscles using induction batteries. Toward the end of the rest cure, patients were gradually allowed to sit up and leave the bed more and more often until they were ready to return home to their ordinary lives.

From Mitchell's own account, the emphasis appears to be on rest and diet. However, an examination of what happened to some of his patients reveals a hidden agenda. Charlotte Gilman was in an unhappy marriage and was experiencing profound depression (Lane, 1990). She was able to function a little outside of the home while working on feminist causes, but at home, with a husband and daughter to take care of, she became depressed and lethargic. Mitchell felt that these women had become selfish and needed moral education. His goal was to return them to their homes where they could function as good wives and mothers. The notion that the traditional female role in the society might be in need of change never occurred to him. In his clinic, he enforced the cultural norms for women - passivity and acceptance of the commands of men. He sent Charlotte Gilman home with orders to live life as domestic as possible, to have her child with her at all times, to lie down for an hour after each meal, and to never touch a pen, brush or pencil as long as she lived. Charlotte Gilman left Mitchell's clinic after her "cure" and returned home where she came near to a complete psychiatric breakdown which she resolved by leaving her husband and going to California with her daughter where she developed into a major feminist and socialist writer and speaker.

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Morita Therapy

Morita therapy was formulated by Shoma Morita (1853-1923) for treating particular forms of neurosis, particularly neurasthenia (insomnia, fatigue, anxiety and inferiority feelings) and phobias. The goals of Morita therapy are to free patients from their symptoms, to make them more adaptable to their environment, and to engage in constructive activity to build up their self-esteem and self-confidence. The underlying philosophy is Zen Buddhism, namely to have patients admit the true nature of their symptoms, to accept them (Zen indifference), and to behave constructively despite them.

In Morita therapy (Reynolds, 1976), patients stay in the clinic where for a least a week they are confined to bed. They can leave their beds only for washing and the toilet. They are allowed no reading, visitors, radio or television, smoking, writing, talking or working. They are encouraged to think, especially about their problems. The goal is to make them so bored that they become anxious to get up and do *something* despite their symptoms. In the next week, they are allowed up and to walk around the clinic. They may observe the world, keep a diary (in which the therapist writes comments which encourage the patients to accept their feelings and never go against them) and to read aloud from classical works. In the third week, patients help out around the clinic, fixing the buildings and keeping the grounds beautiful. Finally, the patients are allowed to go back in the world by day, returning to their job or schooling, before finally leaving the clinic.

Commentators on Morita therapy have noted that the doctor-patient relationship is crucial (Kora, 1965; Iwai and Reynolds, 1970). The doctor is coercive, benign and authoritarian. The doctor, who is rational, tells his irrational patients what they ought to do. It works best with a self-effacing individual who want to please the

doctor and so "sees the light" and experiences the "correct" thoughts and feelings. Most commentators feel that Morita therapy is ideal for Japanese patients but would not be as effective with typical American patients.

Discussion

Both Mitchell's rest cure and Morita therapy have the effect of returning people who are distressed and dissatisfied with their lives to their conventional roles. There is no consideration given to the possibility that their roles might be unsuitable for them or that their environment contains stressors which ought to be modified. Both systems of treatment stress conformity.

Mitchell's rest cure in the 19th Century was most effective with women who, by modern standards, were clearly oppressed by the dominant male subculture. Mitchell, a conforming member of this dominant subculture, got these women to return to their constricted roles and be relatively more content with the roles than hitherto.

Morita therapy takes patients who live in a culture which stresses conformity and who lack self-confidence and gets them to continue with their roles despite their symptoms of distress. The therapist is obviously a member of the prevailing culture and makes no effort to lead the patient to question the appropriateness of his roles or the value of conforming to the culture.

Thus, both Mitchell's rest cure of the 1800s and Morita therapy of the 1900s take oppressed individuals who are in psychological distress and make them less distressed by their oppression.

References

Chang, S. Morita therapy. *American Journal of Psychotherapy*, 1974, 28, 208-221.

Earnest, E.P. *S. Weir Mitchell, novelist and physician*. Philadelphia: University of Pennsylvania Press, 1950.

Iwai, H., & Reynolds, D. Morita psychotherapy. *American Journal of Psychiatry*, 1970, 126, 1031-1036.

Kora, T. Morita therapy. *International Journal of Psychiatry*, 1965, 1, 611-645.

Lane, A.J. *To Herland and beyond*. New York: Pantheon, 1990.

Martin, F.H. Hystero-neurasthenia, or nervous exhaustion of women, treated by the S. Weir Mitchell method. *Journal of the American Medical Association*, 1887, 8, 365.

Mitchell, S.W. *Fat and blood*. Philadelphia: Lippincott, 1877.

Reynolds, D.K. *Morita psychotherapy*. Berkeley: University of California, 1976.

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A three day conference on the current status and future challenges of family-based treatment research will be held on May 5, 6, & 7, 1995, at Temple University, Philadelphia, PA. Titled "Marital and Family Therapy and Process and Outcome Research: The State of the Science", the conference will feature twenty nationally known family researchers who will present and discuss with the audience the theoretical, methodological, statistical, political and practical future of marital and family treatment research. Seventy five people (from all professional levels including students) will be invited to participate. To apply, send a vita, representative publications, and a one page application letter by February 15th to Guy Diamond, Center for Research on Adolescent Drug Abuse, Counseling Psychology, Temple University, 249 Weiss Hall, Philadelphia, PA 19122.

Four student scholarships (which provide all expenses related to the conference) will be granted two promising future scholars based on a five page abstract of completed work relevant to this topic. Poster presentations are also invited (follow-APA conference guidelines).

The conference is sponsored by the APA Division of Family Psychology (43) with funding from the APA Science Directorate, and co-sponsorship by Temple University, The Philadelphia Child Guidance Center, and the APA Division of Independent Practice (42).